

A FREE GUIDE FOR CASC CANDIDATES

The Examiner's Briefing

What we are actually looking for in the CASC.

The skills that decide the exam, from the other side of the table.

The CASC is not a knowledge exam. It is judged on how you communicate, structure your thinking, and carry yourself with a patient. This briefing sets out what an examiner is weighing while you speak — and where good doctors quietly lose marks.

WHAT'S INSIDE

- How the CASC is actually marked — and the two bars you must clear
- The errors examiners see most, and the lines you cannot leave out
- The grammar of a British consultation, and the patient who will not be managed

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How to read this briefing

This is not a revision guide, and it is not a list of stations to memorise. It is something you have almost certainly never had access to: a clear account of how the CASC looks from the other side of the table — what an examiner is weighing, what reassures them, and where they watch good doctors quietly lose marks.

Most CASC preparation is written by people who have recently sat the exam. That has value, but it can only ever describe the view from the candidate's chair. This briefing is written from the examiner's chair — by a consultant forensic psychiatrist who marked the exam against the very standard you are trying to meet. Read it in one sitting; it is meant to reframe how you think about the CASC, not to bury you in detail.

FROM BEHIND THE TABLE · WHAT THIS BRIEFING DOES, AND DOES NOT, DO

It gives you the map — the marking logic, the anatomy of a station, and the errors we see most often. The map matters enormously, because most candidates are working without one.

What it cannot do is build the skill. Communication, structure and presence under pressure are performance skills. You build them by doing them and being corrected — which is exactly what the Academy's stations and practice cards are for. Wherever this briefing reaches the edge of what a map can do, it will say so plainly.

CHAPTER 1

The gap that fails good doctors

Most candidates who fail the CASC are not short on knowledge. They fail on what they do in the room — and they never quite saw it coming.

By the time you sit the CASC, you have already proven what you know. The written papers tested your knowledge and you passed them. So it is worth saying clearly, at the outset, what the CASC is actually for: it is not a knowledge exam wearing a different hat. It is an assessment of clinical skill — of how you gather information, how you communicate, how you reason under time pressure, and how you carry yourself with a patient.

This is the gap that catches strong doctors. They prepare for the CASC the way they prepared for the papers — by reading more, knowing more, revising more. And then they walk into a station and the marks turn on something the reading never touched: whether the examiner believed in them.

The question behind every mark

There is a single question an examiner is, in effect, asking themselves throughout your station. It is not written on any marksheet, but it sits underneath all of them:

THE QUESTION BEHIND EVERY MARK

“Would I be confident to have this candidate as my registrar — someone I could hand a patient to, and trust to manage them safely?”

Hold that question in mind, because it explains everything that follows. The exam is benchmarked against a specific, knowable standard — the competence expected of a newly appointed senior trainee, ready to step up. Not perfection. Not brilliance. A safe, structured, trustworthy colleague. Every domain you are scored on is really a facet of that one judgement. When you understand that the examiner is deciding whether they would trust you with their patients, you stop performing for an exam and start demonstrating that you are safe to delegate to — which is the thing that actually passes the CASC.

IN THE ACADEMY

That sentence opens every one of the forty-three stations in the Academy. It is the lens each station is marked through, and by the end of the library you will have applied it to yourself several hundred times.

CHAPTER 2

How we actually mark you

The marking is more forgiving, and more unforgiving, than most candidates assume. Understanding it changes where you should spend your energy.

You do not need to master the statistics behind CASC marking, but you do need to understand its shape, because most candidates misjudge it — and misallocate their effort as a result.

Two bars, not one

To pass, you must clear two separate bars. The first is an overall standard across the whole exam. The second is a minimum number of individual stations passed. You must clear both. This has a consequence that catches people out every day: a few outstanding stations cannot rescue you if you fail too many others. There is no banking of brilliance. A run of strong performances does not buy back a cluster of weak ones.

FROM BEHIND THE TABLE · WHERE THIS SHOULD CHANGE YOUR PREPARATION

Because you cannot compensate, your weakest station types are worth far more of your attention than your strongest. Most candidates do the opposite — they rehearse the stations they already enjoy, because it feels good, and avoid the ones that frighten them.

The candidate who reliably passes is rarely the one with the highest peaks. It is the one with no troughs. Spend your energy raising the floor, not the ceiling.

How a single station is judged

Within a station, the examiner forms a view across several domains — your clinical skill, your reasoning, your communication, your professionalism — and an overall judgement of your performance. Those two kinds of mark work together to set where the bar sits. What matters for you is simpler: the examiner is building an impression across the whole seven minutes, and the domains are the language they use to describe it. A station is not a checklist to be cleared item by item. It is a performance to be judged as a whole.

IN THE ACADEMY

The Academy is organised the way examiners think: eight domains, each with the stations that test it, and a domain tracker in the workbook so that you can see — in the examiner's own categories — where your floor actually is. The exam's current format and timings change from time to time; check them against the College's published information before you sit.

CHAPTER 3

The anatomy of a station

Every station is engineered. Once you can see how it is built, you can read what it wants from you in the first few seconds.

A CASC station is not an open-ended clinical encounter. It is a designed instrument, built around a specific thing it is trying to assess. Behind every station sits a clear definition of what a competent performance looks like — and the examiner is marking you against that definition, not against an idealised, textbook consultation. Knowing this is oddly freeing: it means the station is bounded, and your job is to find the boundary.

Read the instructions as a map of the marks

Your instructions before each station are not throat-clearing. They tell you, often quite directly, where the marks are. They set the scene and then specify what you are expected to do — and, crucially, sometimes what you are not expected to do. These negative instructions are the most

commonly missed lines in the entire exam. A candidate who chases a thread the station has explicitly told them to leave alone is not being thorough; they are spending their seven minutes in the wrong place.

The role-player is responding to your skill

The person opposite you is a trained professional working to a brief, and that brief is written to react to how you conduct yourself. Handle them with warmth and structure and they will open up. Handle them poorly — rush them, talk over them, miss their distress — and they are instructed to respond as a real patient would, with the guardedness or irritation you have earned. The station is reactive, not scripted. This is why technique is not decoration on top of knowledge; it is the thing that determines what information you are even given.

FROM BEHIND THE TABLE · THE CLEAREST SIGNAL WE SEE

The skilled candidate seems to draw the history out effortlessly, because the role-player is meeting them halfway. The struggling candidate appears to be interrogating a reluctant witness — and does not realise they created that reluctance themselves.

IN THE ACADEMY

Every practice pack in the Academy is written this way. The role-player card carries hidden information and the exact conditions for releasing it — and the lines to say when the candidate earns guardedness instead.

CHAPTER 4

Finding the theme

Range, depth, and the seven-minute discipline. This is the single most important chapter in this briefing — almost everything else serves it.

Let us be honest about the time. No one — not the College, not the examiner — believes you can perform a complete, comprehensive psychiatric assessment in seven minutes. You could not do it in clinic either. So the exam does not ask you to. This is the reassurance that should lower your pulse: the station is not an impossible race to cover everything. It is a test of whether you can find the right things and serve them well.

Every station has a theme

Because a station cannot open endless avenues, it does not. There is always a dominant theme — occasionally two — and everything else in the scenario feeds into it. Your central task in the reading time is to identify that theme, and then to spend your seven minutes in proportion to it.

The candidate who fails this is usually not lazy or ignorant. They are the one who gets pulled into rich but low-value background — a detailed family history, a leisurely social history — while the thing the station was actually built to assess is touched only lightly, or reached only as the bell goes.

Range versus depth

This is the real skill the CASC is testing, and it is a clinical skill, not an exam trick. Knowing what to cover broadly and what to drill into deeply — when to open out and when to focus — is exactly the judgement that makes a doctor safe under time pressure. An examiner can see it within two or three minutes: the candidate with good range-and-depth control moves with intent; the one without it either skates over everything or sinks into the first interesting detail and never resurfaces.

The lines you cannot leave out

Within that discipline, a few things must be touched even when time is short, because their absence is what makes a candidate look unsafe. Risk is the clearest of them — even a brief, well-placed risk enquiry signals that you think like a psychiatrist. The role of drugs and alcohol, and relevant comorbidity, sit in the same category. An examiner notices the absence of these immediately, because forgetting to ask about risk is precisely what you would not want in a registrar you were about to leave in charge.

FROM BEHIND THE TABLE · THE SIMPLEST STATIONS ARE THE HARDEST

Candidates relax when they draw an apparently simple station — a mental state examination in a first presentation, or giving straightforward information about a common medication. That relaxation is the trap. Difficulty in the CASC is not about complexity; it is about expectation. When a station looks easy, the examiner expects it done cleanly, and the bar for a clear pass quietly rises.

And “simple” is rarely as simple as it looks. Psychiatric phenomena carry risk inside them — certain symptoms point to danger that a candidate who has filed the station under “easy” may walk straight past. The station that frightens you is often more forgiving than the one that reassures you.

IN THE ACADEMY

Every station in the Academy is built around one named trap — the specific way the theme gets missed. You read the station failed twice, in two different ways, and then passed, line by line, with what the examiner was thinking at each moment. Then you make the seven decisions yourself, and the examiner tells you why.

The grammar of a British consultation

The marks turn on a style of communication that is rarely taught and easily missed — especially by excellent doctors trained in a different system.

This chapter matters most if English is not your first language, or if you trained outside the United Kingdom — though every candidate should read it. The CASC rewards a particular consultation style, and much of it is invisible until someone names it for you.

The accent is not what is being assessed

Let us deal with the most common and most damaging worry first. Many international candidates believe their accent is a problem to be hidden. They become so preoccupied with concealing it that they stop listening properly and lose their fluency — and that is what costs them, not the accent. Hear this clearly, from the examiner's side of the table: your accent is not being marked. Clarity is. Active listening is. A doctor who speaks with an accent but communicates clearly and listens genuinely scores well. The energy you spend fighting your own voice is energy taken from the patient in front of you. Let the accent be; tend to the communication.

Empathy has to be felt, not signalled

Empathy is one of the most heavily weighted things in the room and one of the most frequently faked. An examiner and a trained role-player can both tell, almost instantly, the difference between empathy that is felt and empathy that is performed. The candidate who delivers a rehearsed line of concern while their eyes are on the clock sends a worse signal than one who says nothing. The small misjudged habits give it away — the reflexive “okay” dropped on top of something painful a patient has just disclosed, as if to move them along, when what the moment needed was for you to stop and stay with it.

The body says it too

Communication is not only what you say. How you sit, what your face does, when a smile warms the room and when it misreads it — all of this is being taken in. So are the tells of insecurity: glancing at your watch, looking rushed, or repeatedly checking the examiner's face for a flicker of approval. A registrar you would trust does not look to you for reassurance in the middle of seeing a patient. The candidate who keeps seeking the examiner's eye is, without meaning to, undermining the very impression of competence they are trying to create.

Their agenda, not only yours

When a station asks you to give information or to address a concern, the work begins before you explain anything: understand what the person already knows, and why they have come today. There is usually one question that, if they left without an answer to it, would make the whole encounter feel like a failure to them. Find that question. A consultation built around the patient's real agenda, rather than your own checklist, is the heart of what the CASC calls patient-centred — and the opposite of the tick-box approach that examiners read as hollow within moments.

Closing well

Two habits separate a controlled finish from a ragged one. The first is checking understanding — not assuming your explanation landed, but confirming it did. The second is a brief safety-net close: a simple “is there anything you were hoping I would ask that I haven't?” or “have I answered what you came in for today?” Used with genuine attention, it signals control and can recover a thread you missed. Used as filler, because you have run dry with two minutes left, it rings as empty as it is. The tool is only as good as the intent behind it.

FROM BEHIND THE TABLE · ON SUMMARISING

Many candidates love to summarise, and a good summary is a genuine asset — brief, focused and purposeful, it shows the examiner you have gathered and organised what you heard. But the same act sends the opposite signal when it is used to fill time at the end because you have nothing left to say. Same words; opposite message. The examiner can tell which one they are watching.

IN THE ACADEMY

Every one of these moments is on the page in the Academy, in the words it was said in — the mistimed “okay”, the felt-empathy pause, the controlled close — with the examiner's thought beside it. And then the practice cards put you in the chair, with a colleague marking you against the same domains.

CHAPTER 6

The errors we see most

None of these are about knowledge. All of them are fixable. Most candidates commit at least one without ever knowing it. Read them as a self-audit — the Error Log that accompanies this briefing turns them into one.

Loss of structure. The candidate jumps from point to point until the thread is lost and the examiner can no longer follow where the consultation is going. It is the most common single failing, and it reads as a mind that is not organised under pressure.

Being unheard or unfollowable. A voice too quiet to catch, or a pace too fast to track. It is mechanical, it is entirely fixable, and candidates almost never realise they are doing it. If the examiner is straining to hear or to keep up, your content barely matters.

Drifting off the theme. Getting pulled into low-value background while the thing the station was built to assess is left thin. Covered fully in Chapter 4 — and worth naming again here, because it is where structure and theme failures meet.

Mistiming the clock — in both directions. Running out of time is an obvious fault. Finishing very early is the one candidates underestimate: an awkward silence with two minutes left tells the examiner you ran out of things to explore, which is rarely because there was nothing left to find.

The tick-box consultation. Working through a memorised list rather than responding to the person in front of you. It is the opposite of patient-centred, and it is unmistakable from the table.

Empathy that does not land. Performed concern, the misjudged “okay”, words of warmth delivered while plainly elsewhere. Examined in Chapter 5; listed here because it sinks more stations than candidates believe.

The professionalism lapse. Rudeness, dismissiveness, a loss of respect or composure with a difficult patient. This is the gravest error of all: a serious enough lapse can sink an otherwise passing day, because it speaks directly to whether you are safe to trust with patients.

The high-pressure patient

One archetype defeats more checklist-trained candidates than any other: the difficult or distressed patient who presses, again and again, for an answer you cannot give them in the way they want it. Consider the patient with erotomania who will not let go of the one demand — aren't you going to let me see her? — and returns to it no matter where you steer. The station is not testing whether you know what erotomania is. It is testing whether you can hold the relationship and stay safe while declining to collude, under sustained pressure, without becoming cold or losing your structure. The candidate with only a list of facts has nothing to reach for when the patient refuses to follow the list.

FROM BEHIND THE TABLE · WHY THIS IS THE TEST THAT SORTS THE FIELD

Anyone can take a history from a co-operative patient. The CASC deliberately includes encounters that push back — because real psychiatry does. How you manage the patient who will not be managed is the clearest evidence an examiner gets of whether you could hold your own as their registrar.

IN THE ACADEMY

That patient is Station 3.7 in the Academy. You read the candidate who argues with him, the candidate who colludes with him, and the candidate who holds the line — and then the practice pack puts you opposite him.

CHAPTER 7

On the day — and in your head

Composure is a skill too, and an examinable one. The candidate who manages their own mind has an advantage that knowledge cannot give.

The unfamiliar station is where temperament shows most. Sooner or later you will face a scenario you have never practised, and many candidates feel the floor drop. Here is the reframe that should steady you: a station you have never seen is one you are expected to find challenging, which means the standard for it is forgiving. The real test is not familiarity — it is composure. And you are not starting from nothing. You are a psychiatrist. You have met strange, difficult, unfamiliar presentations before, and you will again across your career. Ask yourself the only question that matters: in a real clinic, how would I begin with this patient? Then begin there. Your clinical instinct is more reliable than any script, and it does not desert you just because the scenario is new.

FROM BEHIND THE TABLE · THE MINDSET THAT CARRIES YOU THROUGH THE CIRCUIT

When you leave a station, leave it. Close the door in your mind and give the next one a clean start. The structure is built to allow this — stations stand on their own, and a weak one is genuinely survivable, so carrying it with you can only cost you the next.

Do not punish yourself between stations. Self-flagellation is exactly how one shaky station becomes three. Be as steady with yourself as you would want a colleague to be.

And the unglamorous foundations: sleep properly the night before, dress professionally, and let yourself look as confident as you can. Looking composed helps you feel composed — and the examiner is reading your composure from the moment you walk in.

CHAPTER 8

What “good” looks like

You can read every principle in this briefing and still never have done one. That gap is the whole point.

Picture two candidates opening the same station. Both know the medicine. The first sits down, settles the patient with a sentence, signals where the next few minutes are going, and begins

with an open question — then narrows with intent as the picture forms. Their structure is visible without being rigid. When the patient discloses something painful, they stop, and stay with it, before moving on. The examiner relaxes: this is someone they could trust.

The second candidate knows exactly the same facts. But they open without orienting the patient, reach for a remembered checklist, talk a little too fast, glance at the clock, and meet a moment of distress with a brisk “okay” and the next question. Nothing they do is a knowledge error. And yet the examiner’s confidence has already started to drain.

That difference — the entire difference between a pass and a fail in many stations — is a matter of what was said, in what order, at what moment. You have just read a description of it. What you have not done is the thing itself: chosen the sentence at the moment the patient went quiet, felt the pull towards the checklist and resisted it, and been told afterwards, by someone who was marking you, exactly where the mark was won. Watching a good candidate teaches you what good looks like. Failing a station safely, and then passing it, teaches you how to do it.

THE NEXT STEP

This is where The CASC Academy begins. Forty-three stations across the eight domains, each built around one named trap. In every one you read the station failed twice and passed once, line by line, with the examiner’s reasoning at each turn; you make the seven decisions yourself; you sit it under time on a patient you have not met; and you take the practice cards to two colleagues that evening. The workbook plans your weeks, and the candidates’ WhatsApp group finds you the two colleagues.

One station is open to anyone, with no login. Start there.

MEET YOUR INSTRUCTOR

Dr Ahmed Abouelghit

UK-trained Consultant Forensic Psychiatrist · former CASC examiner



Dr Ahmed Abouelghit is a UK-trained Consultant Forensic Psychiatrist who examined the CASC for the Royal College of Psychiatrists before stepping down to teach candidates. Across a career spanning the United Kingdom, Egypt and Qatar, teaching has been at its centre: he has spent years preparing, supervising and examining the doctors who sit this exam.

As an educational supervisor and training programme director he leads the structured teaching and assessment of psychiatric trainees — the close, candid feedback that turns a competent doctor into a confident one. It is the same examiner's-eye feedback this briefing, and every station in the Academy, is built on.

His senior clinical and medico-legal experience, including work connected to mental health tribunals and the courts in the UK, informs one principle that runs through all his teaching: clinical reasoning must be clear, structured, and able to withstand scrutiny. That is exactly what the CASC is testing.

He is the founder of MedLex, through which he brings frontline experience and examiner-level insight into serious, practice-grounded education for clinicians.

THE PERSPECTIVE BEHIND THIS BRIEFING

He has sat where you are sitting now — passing candidate, senior registrar, consultant, and, until he stepped down to teach candidates, examiner. This briefing distils that arc.



BEFORE YOU GO

You understand the exam. Now fail a station safely.

One station of the Academy is open to anyone, with no login. Read it fail twice, read it pass, tap the examiner's reasoning, and make the seven decisions yourself.

[Open Station 1.2 — free →](#)

medlexsolutions.com/casc-academy

YOUR FREE COMPANION TOOL

The Examiner's Error Log

A one-page self-audit of the errors in Chapter 6 — score your own practice, station by station, from the examiner's side of the table. Sent with this briefing.

[Open the Error Log →](#)

The CASC Academy: 43 stations across 8 domains, each with two failing takes and a passing one, the examiner's reasoning at every decision, a timed Exam Mode and a three-person practice pack.

The CASC Academy is an independent educational course produced by MedLex Foundations. It is not affiliated with, endorsed by, or accredited by the Royal College of Psychiatrists. All patients described are fictional composites.

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